SACTCHECK · TREATMENT PASSPORT

Phesgo + vinorelbine

A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Phesgo on Day 1; vinorelbine on Days 1 and 8 of each 21-day cycle. Vinorelbine is prescribed by intravenous (IV) OR by mouth, not both. There is no planned Day 15 vinorelbine. Continue while beneficial and tolerated; any antibody-only phase is separately prescribed.

The four pages are reusable recording sheets, not a prescribed four-cycle course. Ask for additional sheets as needed.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00798/

Full patient information, sources and visual timeline. No identifiers are encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

Phesgo + vinorelbine

Cycle number: _____ Start date: _____ Next review: _____

Current regimen · reusable cycle record · copy this sheet for another cycle of the same phase.

Pertuzumab · HER2-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/day or at night: call	□ 7+ extra stools/day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Trastuzumab · HER2-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
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Vinorelbine · Vinca-alkaloid chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Constipation	□ Usual bowel pattern	□ Mild change: follow the agreed bowel plan	□ Persistent constipation or discomfort: contact the team	□ Pain, vomiting or no stool/wind: urgent assessment	□ Collapse or severe sudden illness: emergency help
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Phesgo + vinorelbine

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